

# HOW TO OPEN A HOME CARE AGENCY IN NORTH CAROLINA

A North Carolina-licensed Home Care Agency — delivering non-medical in-home aide services (hands-on help with daily living), companion, sitter, and respite services, and home management, so seniors and adults with disabilities can live safely and independently at home.

## A step-by-step startup and operations guide

Licensing (DHSR under 10A NCAC 13J) · Screening & training · Getting paid · Operations

---

**Regulatory basis:** The Rules Governing the Licensing of Home Care Agencies, 10A NCAC 13J, under N.C.G.S. §§ 131E-135 through 142; licensed and overseen by the North Carolina Division of Health Service Regulation (DHSR), Acute and Home Care Licensure and Certification Section, with rulemaking by the NC Medical Care Commission. This is a NON-MEDICAL license — in-home aide personal care, companion/sitter/respite, and home management, but not skilled nursing or certified home health.

No Certificate of Need is required for non-medical home care. Funding: private pay; North Carolina Medicaid Personal Care Services and the Community Alternatives Program waivers (CAP/DA, CAP/C) where applicable; Veterans' benefits; and long-term-care insurance.

---

## How to Use This Guide

This guide walks you from 'I want to start a home care business' to 'I am licensed, serving clients, and getting paid' in North Carolina. The big picture is simple: in North Carolina, non-medical home care is delivered by a Home Care Agency licensed by the Division of Health Service Regulation (DHSR) under 10A NCAC 13J. North Carolina requires this license for all home care — including purely non-medical personal care and companion services. Unlike a Medicare-certified home health agency, a non-medical home care agency does NOT need a Certificate of Need, which makes it a far faster, lower-cost way into the state's senior-care market. Your agency can provide real hands-on personal care — help with bathing, dressing, and other activities of daily living — alongside companionship, sitter, respite, and home management. Skilled nursing (injections, wound care, medication administration) stays outside your scope; that is home health, a different and much heavier license that does require a CON.

Read Parts 1 through 4 before you spend real money — they tell you what you are allowed to do and exactly how to get licensed, including North Carolina's distinctive Nurse Aide Registry rule and its 90-minute service-area limit. Parts 5 and 6 are about revenue: where clients come from and how you get paid, including North Carolina Medicaid's Personal Care Services and its multi-vendor Electronic Visit Verification. Parts 7 through 9 keep you compliant once you are open. The two worked examples show how it comes together in a metro market, and the contacts, glossary, and references at the end are your quick-reference desk. Verify every fee, figure, and phone number before you rely on it — requirements change.

### **The one-paragraph version**

Form your business with the NC Secretary of State and get an EIN and insurance. Decide your services (in-home aide personal care and/or companion, sitter, and respite). Line up a registered nurse to assess clients and supervise personal care. Apply for a Home Care Agency license with DHSR under 10A NCAC 13J; complete the required training if you have never operated a home care agency, and run criminal-history and Health Care Personnel Registry checks on your administrator, owners, and every caregiver before client contact. Pass your initial (unannounced) licensure survey. Bill private-pay clients from day one; to bill Medicaid, enroll through NCTracks, deliver Personal Care Services under Clinical Coverage Policy 3L, and clock every visit through the right EVV vendor (Sandata for NC Medicaid Direct; HHAeXchange for managed-care plans). Stay in scope, supervise every client at least every 90 days, and document everything.

## The Path at a Glance

1. **Form your business** — register your LLC or corporation with the NC Secretary of State, file any assumed business name (DBA) with your county Register of Deeds, and get your EIN.
2. **Decide your services and service area** — in-home aide (personal care) and/or companion, sitter, and respite; define your county, contiguous counties, or 90-minute service area.
3. **Line up your registered nurse** — an RN (or other appropriate professional) to assess clients, write plans of care, and provide supervision for personal care.
4. **Prepare policies and records** — a 10A NCAC 13J-compliant policy manual, client and personnel records, and your statistical records.
5. **Complete required training** — if you have never owned or operated a home care agency, complete the DHSR-approved training before licensure.
6. **Screen your people** — criminal-history checks (§ 131E-265) and Health Care Personnel Registry checks (§ 131E-256) before any client contact.
7. **Apply to DHSR and pass your survey** — submit your application and fee, then pass the unannounced initial licensure survey to earn your license (complete within 12 months).
8. **Start serving private-pay clients** — begin revenue immediately while you build Medicaid capability.
9. **Add Medicaid** — enroll through NCTracks, deliver PCS under Policy 3L (and CAP/DA, CAP/C where applicable), and set up EVV.
10. **Operate and stay compliant** — supervise every client at least every 90 days, keep screening and training current, and document everything.

## Part 1 — What This Business Is

In North Carolina, a Home Care Agency is a private organization that provides home care services for pay to people in their residences. Under 10A NCAC 13J and N.C.G.S. §§ 131E-135 through 142, the license is issued by service category. Your non-medical business provides in-home aide services (personal care and home management) and companion, sitter, and respite services, and leaves skilled nursing and certified home health out. You are licensed and surveyed by the Division of Health Service Regulation (DHSR), Acute and Home Care Licensure and Certification Section, and the North Carolina Medical Care Commission writes the rules.

The most important structural fact for a new owner: a non-medical home care agency does NOT require a Certificate of Need. In North Carolina, CON is required to open a Medicare-certified home health agency or a hospice — a slow, competitive, expensive process. Non-medical home care skips it entirely. That is why so many entrepreneurs start with a home care agency license and add certified home health later, if ever.

### **The non-medical line — know it cold**

Your in-home aides may help with bathing, dressing, mobility, eating, toileting, meals, and homemaking, and may **assist** a client with self-administration of medication (reminders, bringing the container). They may NOT administer medications, give injections, or do wound care — those are skilled nursing tasks that require a home health license. Cross that line and you are operating outside your license.

One North Carolina wrinkle to understand from day one: the Nurse Aide Registry rule. If a client's plan of care requires the aide to provide extensive hands-on assistance, that aide must be listed on the North Carolina Nurse Aide Registry (Nurse Aide I). If the plan requires only limited assistance, Registry listing is not required. You will match each client to an appropriately qualified aide — more on this in Part 3.

It helps to understand why the market is strong. North Carolina is aging fast — there are now more North Carolinians over 60 than under 18 — and the overwhelming preference among older adults is to stay in their own homes rather than move to a facility. That preference, combined with a Medicaid program that funds personal care at home and a documented, growing shortage of home-care workers, means demand for reliable non-medical home care is high and rising across the state's metros and rural counties alike. A well-run agency that screens carefully, supervises on schedule, and shows up reliably is filling a real and durable need.

Think of your license as issued by service category. Many new owners begin with in-home aide services (which includes hands-on personal care) plus companion, sitter, and respite — that combination covers most of what families ask for. You can start with a narrower set and add a category later by giving DHSR written notice at least thirty days before you begin providing it. Whatever categories you hold, the boundary is the same: everything you do is non-medical, and skilled nursing is out.

## Part 2 — Getting Licensed with DHSR

You apply for your Home Care Agency license through DHSR's Acute and Home Care Licensure and Certification Section (2712 Mail Service Center, Raleigh NC 27699; 919-855-4620; DHSR.homecare@dhhs.nc.gov). Each site that provides home care services is separately licensed. Submit your application with the required fee, and be prepared to complete every requirement within twelve months of your application-and-fee date, or you start over.

### **First-time owners: required training**

If you have never owned or operated a home care agency, DHSR will not issue your license until you complete the DHSR-approved training covering the licensure requirements, the licensure process, and the rules for operating a home care agency. Build this into your timeline early.

After your application is accepted, DHSR conducts an initial licensure survey — an on-site review of your policies, records, and readiness. This survey is unannounced, as are all later inspections and complaint investigations. DHSR's own guidance notes that the main reason applicants are not approved is that they did not understand or comply with the initial survey checklist and the licensure rules, so prepare against that checklist before you invite

the survey. Once you pass, you receive your license; all home care licenses expire at midnight on December 31, and you renew annually before then.

#### **A note on the fee**

Published figures for the North Carolina home care license fee vary across third-party sources (you will see numbers ranging roughly from \$400 to \$560). Because the sources disagree, do not budget from them — confirm the current application and renewal fee directly on the DHSR home care licensure application and fee schedule before you apply.

What does DHSR actually look for at the survey? In broad terms: that your policies are written and implemented, not just sitting in a binder; that your client records contain signed plans of care, assessments, and evidence of supervisory visits; that your personnel records show completed criminal-history and Health Care Personnel Registry checks, documented competency, and, where required, Nurse Aide Registry listing; and that you maintain the administrative and statistical records the rules require. Walk your own operation against the initial survey checklist before you invite the surveyor, and fix gaps in advance rather than explaining them on the day.

Plan your timeline realistically. Business formation and insurance take a week or two. Writing or adopting a compliant policy manual, building your record systems, and completing first-time-owner training take longer. Screening and hiring your first caregivers runs in parallel. Once your application and fee are in, you have twelve months to complete everything and pass the survey — generous, but the clock is real, so keep momentum. Many owners find the survey itself is the easy part if the preparation was thorough.

#### **Accreditation is optional**

You are not required to be accredited to hold a North Carolina home care license. Some agencies pursue accreditation later — for example when they add Medicare-certified home health — because payers or referral partners value it, but it adds cost and is not part of the non-medical licensure path. Start with the license; consider accreditation only if and when your business model calls for it.

### **Part 3 — Background Screening & Caregiver Qualifications**

Before anyone has contact with a client, North Carolina requires you to run a criminal history record check under N.C.G.S. § 131E-265 and to check the North Carolina Health Care Personnel Registry under N.C.G.S. § 131E-256 for the administrator, all owners, and every direct-care employee. The Health Care Personnel Registry records substantiated findings of abuse, neglect, or misappropriation of property; a person with such a finding, or a disqualifying criminal history, does not serve your clients.

#### **The Nurse Aide Registry rule (the part people miss)**

North Carolina ties aide qualifications to the level of assistance the client needs. Under 10A NCAC 13J .1102 and .1202, if a client's plan of care requires the in-home aide to provide extensive assistance (as defined in the rule), that aide must be listed on the North Carolina Nurse Aide Registry as a Nurse Aide I — competency is established through the North Carolina Board of Nursing. If the plan requires only limited assistance, the aide need not be on the Registry. Practically: check each client's plan of care before you assign an aide, and staff extensive-assistance clients only with Registry-listed aides.

Regardless of Registry status, you must document that every aide is competent for each task assigned — competency includes a demonstration of the task to the appropriate professional — and you prepare aides consistent with the North Carolina Home Care Aide curriculum. Companion, sitter, and respite staff (non-hands-on) have lighter requirements but still must have documented competency for their tasks.

A word on the levels of assistance, because they drive your staffing. North Carolina rates a client's need for each activity of daily living on a scale from totally independent, through needing cueing or supervision, to needing limited hands-on assistance, to needing extensive hands-on assistance, to totally dependent. The professional assessment (Part 4) determines where each client falls, and that determination flows into the plan of care — which in turn tells you whether the assigned aide must be Registry-listed. This is why the assessment and plan of care

come first and staffing follows: you cannot know which aide is appropriate until you know the level of assistance the client needs.

### Who leads the agency

Every agency designates an Agency Director with administrative responsibility for the operation. For an agency licensed to provide companion, sitter, or respite services only, the director's qualifications are lighter — a high school diploma or GED plus one of the qualifying criteria in the rule. For an agency providing in-home aide personal care, professional supervision is provided by the appropriate professional (your RN). Document the director's qualifications in your records, and designate someone qualified to act when the director is away, so supervision and availability never lapse.

On employment and pay: as a third-party employer you follow the federal Fair Labor Standards Act and cannot claim the companionship exemption for your caregivers — pay at least the federal minimum wage and overtime after forty hours in a workweek, and track compensable travel between clients. North Carolina has no higher state minimum wage and no state daily-overtime rule. Classify workers correctly as employees rather than contractors; misclassification is a common and costly mistake, and it interacts with your workers' compensation obligation.

## Part 4 — Setting Up to Operate

Form your entity with the North Carolina Secretary of State ([sosnc.gov](http://sosnc.gov)) and file any assumed business name with your county Register of Deeds. Get your EIN, open a business bank account, and put insurance in place — general and professional liability, and, importantly, workers' compensation. North Carolina requires workers' compensation once you have three or more employees (corporate officers count toward the three; sole proprietors, LLC members, and partners are not automatically counted but may elect coverage). The NC Industrial Commission administers workers' comp.

Define your geographic service area now, because North Carolina limits it for in-home aide services: your county, the counties contiguous to it, or the area within 90 minutes' driving time from your site — whichever is greater. The limit exists so your professional supervisor can reach every client's home for the required quarterly supervisory visit. Do not accept in-home aide clients outside that area.

### The 90-day supervision clock

For every client receiving personal care, your appropriate professional (an RN) must make a supervisory visit to the client's home at least every 90 days, reviewing the client's condition and the services provided. That same visit can double as the quarterly case review of the plan of care. Build a tracker from each client's start date so no 90-day visit is ever missed — surveyors check this closely.

Your registered nurse is central to a compliant operation even though you are non-medical. The RN (or another appropriate professional such as a social worker, therapist, or dietitian, depending on the service) conducts the initial assessment that determines each client's level of assistance, writes the plan of care the client signs before services begin, and makes the quarterly supervisory visits. The RN is also continuously available for supervision during the hours your aides are working. You do not need the nurse on-site all day, but you do need a real, reachable professional standing behind the care — so line this up before you take your first personal-care client.

Build your record-keeping to survive an unannounced inspection. For each client: intake and safe-service determination, the professional assessment, the signed plan of care, visit notes that match the plan, supervisory-visit records, and any incident or grievance records. For each employee: application and qualifications, criminal-history and Health Care Personnel Registry checks, Nurse Aide Registry status where required, documented competency, orientation and in-service training, and annual evaluation. For the agency: your policies, statistical records, and (for Medicaid) enrollment and EVV setup. Retain records at least three years. Good records are not paperwork for its own sake — they are how you prove, on any given day, that the care was appropriate and delivered.

## Part 5 — Finding Clients & Referrals

For Medicaid clients, the most important relationship in your region is your Area Agency on Aging (AAA). North Carolina's 16 AAAs sit inside regional Councils of Governments under the NC Division of Aging and Adult Services, and they — along with county Departments of Social Services — are central to how older adults find in-home services. The statewide gateway is the NC CARE-LINE (1-800-662-7030), which routes callers to their local aging resources.

### Referral targets are in your Referral System edition

The North Carolina edition of the Referral System lists real hospitals and discharge planners, skilled-nursing and rehab centers, hospices, assisted living communities, funeral homes, elder-law attorneys, and geriatric care managers — by metro (Charlotte, Raleigh, Greensboro, Durham), with names, addresses, and phone numbers — so you can start outreach immediately. Verify each contact before you rely on it.

Beyond the aging network, your steadiest referral sources are hospital and skilled-nursing discharge planners, geriatric care managers, senior-living communities, and elder-law attorneys — the people families turn to at the moment they realize they need help at home. Build those relationships lawfully (see Part 6 on the anti-kickback line).

The reason referrals matter so much in home care is that care is a trust purchase made at a stressful moment — a hospital discharge, a fall, a diagnosis, a family caregiver burning out. At that moment, people ask someone they trust: the discharge planner, the care manager, the attorney handling a parent's affairs. The agency that professional names is the one that gets the client. So your growth job, especially early, is not 'marketing' in the abstract — it is earning a place on a small number of trusted referral lists and then delivering so reliably that those sources keep sending clients. One discharge planner or one care manager who trusts you can be worth more than an entire year's advertising budget.

Private-pay clients often come from a different direction — adult children researching options online, senior-living communities whose residents need extra in-home support, and word of mouth from satisfied families. A clear website, a Google Business Profile, and a simple one-page leave-behind you can hand to a discharge planner or care manager go a long way. Whatever the channel, your reputation for reliability is your best marketing.

## Part 6 — Getting Paid

You can serve private-pay clients from the day your license is issued — many agencies start there. To bill Medicaid, enroll as a provider through NCTracks (using your NC Medicaid ID and NPI). North Carolina's main non-medical Medicaid benefit is State Plan Personal Care Services (PCS) under Clinical Coverage Policy 3L (HCPCS 99509, taxonomy 251E00000X). A client's need for PCS is determined by an Independent Assessment Entity through NCLIFTSS, and prior approval is issued for a set number of hours. There are also the Community Alternatives Program waivers — CAP/DA for disabled adults and CAP/C for children.

### Electronic Visit Verification — know which vendor applies

North Carolina requires Electronic Visit Verification for Medicaid personal-care visits, and the vendor depends on the payer. For NC Medicaid Direct (fee-for-service), the state's vendor is Sandata — which is also the state aggregator that collects EVV data from every other vendor. For managed-care Standard Plans, Tailored Plans, and LME/MCOs, the free solution is HHAExchange. For Healthy Blue, it is CareBridge. You may use your own compliant EVV system, but you must register it so data flow to Sandata. Capture visits electronically at least 85% of the time, keep manual entries to 15% or less, and make sure your EVV data match the authorization before you bill. Private-pay visits do not require EVV.

The multi-vendor picture confuses many new agencies, so hold onto the simple version: which EVV system you use is decided by who is paying for that particular client, not by your preference. A single agency serving clients across Medicaid Direct and two managed-care plans may touch all three systems. This is why disciplined intake matters —

you need to know each client's payer and plan up front so the right EVV setup and the right authorization are in place before the first visit. When EVV data and claims line up with the authorization, you get paid; when they do not, claims pend or deny. Set this up and test it before you take your first Medicaid client, not after.

### Your funding sources at a glance

Here is the honest map of who pays for non-medical home care in North Carolina. Note that North Carolina Medicaid PCS hours are individually assessed and authorized per client — there is no flat published rate to quote here — so confirm current rates and authorized hours with NC Medicaid and the payer rather than budgeting from any third-party figure.

| Funding source            | What it covers                                                                    | How you access it                                                                     |
|---------------------------|-----------------------------------------------------------------------------------|---------------------------------------------------------------------------------------|
| Private pay               | Any non-medical service, no authorization needed — your fastest revenue           | Your service agreement and schedule of charges; bill the client directly              |
| Medicaid PCS (State Plan) | Personal care (ADL assistance) for eligible adults; hours individually assessed   | Enroll in NCTracks; client assessed by an IAE via NCLIFTSS under CCP 3L; EVV required |
| CAP/DA waiver             | Home & community-based services for disabled adults at risk of institutional care | Community Alternatives Program (Disabled Adults); coordinate with the case manager    |
| CAP/C waiver              | Home & community-based services for medically fragile children                    | Community Alternatives Program (Children); coordinate with the case manager           |
| Veterans' benefits        | In-home support for eligible veterans                                             | VA programs and VA-contracted networks; verify eligibility with the VA                |
| Long-term-care insurance  | Non-medical home care where the policy covers it                                  | Work with the client's policy; provide documentation the insurer requires             |

Most successful agencies start on private pay — it needs no authorization and funds you while your Medicaid enrollment and credentialing move through NCTracks. Add PCS and the CAP waivers as your systems mature, because Medicaid brings EVV, prior authorization, and documentation obligations that reward an agency with disciplined operations.

#### The legal line on referrals

If any part of a client's care is paid by Medicaid, Medicare, or the VA, you may not pay for or reward referrals. The federal Anti-Kickback Statute makes it a crime to offer, pay, solicit, or receive anything of value to induce those referrals, and North Carolina has its own laws against illegal remuneration. Keep every referral relationship legitimate and for fair value — never a per-client fee.

## Part 7 — Your First 90 Days Operating

Your first ninety days set the habits that keep you compliant for years. The goal is to open with a small number of clients served flawlessly rather than a large number served loosely — a clean early record is what carries you through your first survey and builds the referral reputation that grows the business. Here is a workable sequence; adjust the timing to your situation, but keep the order, because assessment and plan of care always precede service, and screening always precedes client contact.

1. Days 1–15: Finalize your policy manual, client and personnel record systems, and your 90-day supervisory-visit tracker. Confirm your RN is ready to assess and supervise.



2. Days 1–30: Complete criminal-history and Health Care Personnel Registry checks on all staff; verify Nurse Aide Registry status for aides who will provide extensive assistance; document competency for every task.
3. Days 15–45: Take your first private-pay clients. For each, complete the professional assessment, write and sign the plan of care before services begin, and schedule the first 90-day supervisory visit.
4. Days 30–60: Enroll in NCTracks and begin Medicaid credentialing; set up your EVV vendor(s) and test visit capture before you rely on them.
5. Days 45–75: Build referral relationships — your Area Agency on Aging, hospital and skilled-nursing discharge planners, and the metro contacts in your Referral System edition.
6. Days 60–90: Run your first internal quality review — check documentation completeness, supervisory-visit timeliness, and EVV capture rates — and fix any gaps before they become survey findings.

## Part 8 — Common Mistakes to Avoid

Almost every problem that delays a license or triggers a survey finding traces back to a handful of avoidable mistakes. None of them are complicated; they are simply the things that slip when an owner is moving fast. Read this list before you open and again before each survey, and build a habit or a checklist around each one.

### The single most common finding

If there is one thing surveyors and consultants see most often, it is a missed or late supervisory visit. The 90-day clock runs from each client's start date, every client is on their own schedule, and it is easy to lose track once you have more than a few. A simple tracker — client, start date, next-visit-due date — that your RN reviews weekly prevents the single most common deficiency in the whole program.

- **Assigning an extensive-assistance client to a non-Registry aide.** Check the plan of care's level of assistance first; extensive assistance requires a Nurse Aide I on the Registry.
- **Missing the 90-day supervisory visit.** This is one of the first things surveyors check. Track it from each client's start date.
- **Starting a client before the plan of care is signed.** The plan of care must be established with the client and signed before services begin.
- **Serving outside your geographic service area.** In-home aide services are limited to your county, contiguous counties, or 90 minutes' drive — whichever is greater.
- **Crossing the non-medical line.** Aides assist with self-administration only — never administer medications, injections, or wound care.
- **Letting a caregiver start on a pending background check.** Screening and Registry checks must clear before any client contact.
- **Guessing at the license fee.** Sources disagree; confirm the current fee on the DHSR schedule before applying.
- **Billing Medicaid without matching EVV.** Your EVV data must match the authorization, routed through the correct vendor, or claims fail.

## Part 9 — Staying Compliant Year-Round

- Make the 90-day supervisory home visit to every personal-care client, and use it as the quarterly case review; document each one.
- Keep criminal-history and Health Care Personnel Registry checks current, and re-verify Nurse Aide Registry status as needed; never let a new hire start client contact on a pending check.
- Renew your license before December 31 every year, and keep your statistical and administrative records (per 10A NCAC 13J .1002) current — retain records at least three years.

- Keep aide documentation and EVV records consistent for every Medicaid client; monitor your manual-entry rate.
- Run regular internal quality reviews and act on what they show — documentation, supervision timeliness, incidents, and grievances.
- Report suspected abuse, neglect, or exploitation of a disabled adult to the county Department of Social Services immediately (call 911 if the person is in immediate danger).

Compliance is not a once-a-year scramble before renewal — it is the daily rhythm of a well-run agency, and it is also your best protection. An agency that supervises on schedule, screens before hire, keeps documentation current, and stays strictly in scope has little to fear from an unannounced survey, because the survey simply confirms what the agency already does. Treat every deficiency, complaint, or near-miss as information: find the root cause, fix it through your quality program, and adjust your training so it does not recur. Agencies that operate this way not only pass surveys — they earn the referral reputation that drives steady growth.

Finally, keep learning. North Carolina updates its rules, its Medicaid policies, and its EVV requirements periodically, and payers issue bulletins that affect how you bill. Stay on the DHSR and NC Medicaid mailing lists, attend the training your Area Agency on Aging and the state offer, and revisit your policy manual at least annually and whenever a rule changes. The regulatory landscape is not static, and the agencies that thrive are the ones that keep current rather than assuming last year's answer still holds.

## Two Worked Examples

The two examples below show the same path playing out in two different North Carolina markets. They are illustrative — the people are invented and every phone number should be verified before you rely on it — but the sequence and the local contacts are real and current. Read them side by side to see what is universal (the license, screening, supervision, and EVV) and what is local (your Area Agency on Aging and your referral partners).

### Worked Example 1 — Metro Charlotte

Danielle wants to open a non-medical home care agency in Mecklenburg County serving metro Charlotte. Here is her path.

1. **Entity & setup:** She forms an LLC with the NC Secretary of State, files her assumed business name with the Mecklenburg County Register of Deeds, gets her EIN, and lines up general/professional liability and workers' compensation coverage (she'll cross the 3-employee threshold quickly).
2. **Service area:** Mecklenburg plus contiguous counties (or 90 minutes' drive) — comfortably covering the Charlotte metro while keeping her RN within reach of every client for the 90-day visit.
3. **RN & policies:** She contracts a registered nurse to assess clients and supervise personal care, and adopts a 10A NCAC 13J-compliant policy manual.
4. **Licensing:** As a first-time owner she completes the DHSR-approved training, runs criminal-history and Health Care Personnel Registry checks on herself and her caregivers, submits her application to DHSR, and passes the unannounced initial survey.
5. **Referrals:** She connects with the Centralina Area Agency on Aging (10735 David Taylor Dr, Suite 250, Charlotte 28262; 704-348-2712), which serves Mecklenburg and eight surrounding counties, plus hospital discharge planners at Atrium Health Carolinas Medical Center (704-355-2000) and Novant Health Presbyterian (704-384-4000), and care managers from her Referral System edition.
6. **Getting paid:** She starts with private-pay clients, then enrolls in NCTracks, credentials for PCS under Policy 3L, and sets up EVV (Sandata for Direct clients; HHAeXchange for managed-care members).

## Worked Example 2 — Raleigh (the Triangle)

Marcus is opening in Wake County, serving Raleigh and the Triangle. His steps mirror Danielle's, tuned to his region.

1. **Entity & setup:** LLC with the NC Secretary of State, assumed business name filed with the Wake County Register of Deeds, EIN, insurance, and workers' comp.
2. **Service area:** Wake plus contiguous counties (or 90 minutes' drive), covering Raleigh, Cary, and much of the Triangle.
3. **Referrals:** He works with the Area Agency on Aging at the Central Pines Regional Council (formerly Triangle J Council of Governments; Region J AAA, 919-558-2708), which serves Wake and the surrounding Triangle counties, plus discharge planners at WakeMed Raleigh (919-350-8000), UNC REX (919-784-3100), and Duke Raleigh (919-954-3000), and geriatric care managers from his Referral System edition.
4. **Licensing & payment:** Same DHSR path — training, screening, application, survey — then private pay first, followed by NCTracks enrollment, PCS credentialing, and EVV setup.

Notice what stays the same across both metros: the license (10A NCAC 13J), the screening (criminal history + Health Care Personnel Registry), the Nurse Aide Registry rule, the 90-day supervisory visit, the 90-minute service-area limit, and the EVV vendors. Only the local Area Agency on Aging and referral partners change.

### How to find your own county's numbers

Wherever you open, you can assemble your local contact list the same way the worked examples do. Start with the NC CARE-LINE (1-800-662-7030) or the Eldercare Locator (1-800-677-1116) to reach your county's Area Agency on Aging — your gateway to the aging network and, for many clients, to PCS and the CAP waivers. Then identify the hospitals and skilled-nursing facilities in your service area and ask each for the case-management or discharge-planning department. Find your county Department of Social Services for both Adult Protective Services reporting and local aging services. Finally, introduce yourself to the geriatric care managers, senior-placement professionals, and elder-law attorneys in your area — they meet families at the exact moment a home-care decision is being made. Your Referral System edition gives you a verified starting list for the four largest metros; build out from there for your own county.

## Part 10 — Growing From Here

Once you are licensed, serving clients, and getting paid, growth is mostly a matter of doing more of what already works — deepening referral relationships, hiring and retaining good caregivers, and keeping your compliance airtight so nothing slows you down. A few directions are worth considering as you scale.

Add service categories or sites deliberately. If you started with companion, sitter, and respite, adding in-home aide personal care opens the larger Medicaid market — give DHSR thirty days' written notice and make sure your RN supervision is ready. If you want to serve beyond your current service area, you can license an additional site, but only if you can still meet the supervision and 90-minute-service-area obligations everywhere you operate. Grow in steps you can supervise, not faster than your quality can follow.

Invest in caregiver recruitment and retention early, because in a tight labor market your ability to staff cases reliably is your real constraint on growth. Agencies that treat caregivers well — fair pay, respect, good scheduling, real support — keep them, and a stable caregiver workforce is what lets you say yes when a discharge planner calls with a new client. Your referral reputation and your staffing capacity grow together.

Some owners eventually add a Medicare-certified home health line — a bigger, clinical business that does require a Certificate of Need and accreditation. That is a different undertaking with a much heavier regulatory load, and it is not necessary; many successful agencies stay non-medical by choice. Whether or not you ever go that direction, the non-medical home care agency you build now is a real, durable business serving a growing need — and it stands on its own.

## Who to Call & What to Ask

| For...                                    | Who to contact                                                                                                                                                                                       |
|-------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Your license (rules, application, survey) | DHSR, Acute and Home Care Licensure & Certification Section — 919-855-4620 · DHSR.homecare@dhhs.nc.gov. Ask: current application & fee, the initial survey checklist, and first-time-owner training. |
| Statewide aging gateway / referrals       | NC CARE-LINE — 1-800-662-7030. Ask for your local Area Agency on Aging and ADRC.                                                                                                                     |
| Nurse Aide Registry / competency          | NC Board of Nursing (ncbon.com) and the NC Nurse Aide Registry. Ask: how to verify and list a Nurse Aide I.                                                                                          |
| Medicaid enrollment & PCS                 | NCTracks (enrollment) and NCLIFTSS/Acentra (PCS assessments). Ask: PCS enrollment steps and CCP 3L requirements.                                                                                     |
| EVV setup                                 | Sandata (NC Medicaid Direct/aggregator) 1-855-940-4915 · HHAeXchange 855-400-4429 · CareBridge 855-782-5976.                                                                                         |
| Business registration                     | NC Secretary of State (sosnc.gov) and your county Register of Deeds (assumed business name).                                                                                                         |
| Workers' compensation                     | NC Industrial Commission (ic.nc.gov). Ask: coverage requirement at 3+ employees and officer inclusion.                                                                                               |
| Reporting abuse of a disabled adult       | Your county Department of Social Services (Adult Protective Services); call 911 if the person is in immediate danger.                                                                                |

## Key Terms at a Glance

- **Home Care Agency license** — the DHSR license under 10A NCAC 13J required for all home care, including non-medical.
- **Certificate of Need (CON)** — required for Medicare-certified home health and hospice, NOT for non-medical home care.
- **In-home aide services** — personal care (ADLs) and home management delivered by trained aides.
- **Nurse Aide Registry (NA I)** — required for aides providing extensive assistance; not required for limited assistance.
- **Plan of care** — the written plan, signed before services begin, stating the level of assistance for each ADL.
- **Supervisory visit** — the professional's home visit to each client at least every 90 days.
- **PCS** — Medicaid State Plan Personal Care Services under Clinical Coverage Policy 3L (HCPCS 99509).
- **EVV** — Electronic Visit Verification (Sandata / HHAeXchange / CareBridge, by payer).
- **HCPR** — the NC Health Care Personnel Registry checked for abuse/neglect findings before hire.

## Core Rules & References

- 10A NCAC 13J — The Rules Governing the Licensing of Home Care Agencies.
- N.C.G.S. §§ 131E-135 through 142 — the home care agency licensure statutes.
- N.C.G.S. § 131E-265 (criminal history) and § 131E-256 (Health Care Personnel Registry).
- N.C.G.S. § 131E-255 — the Nurse Aide Registry.
- N.C.G.S. § 131E-144.3 — Declaration of Home Care Clients' Rights.
- NC Medicaid Clinical Coverage Policy 3L — Personal Care Services.
- N.C.G.S. Chapter 108A, Article 6 — protection of disabled adults (APS).

- N.C.G.S. Chapter 97 — the Workers' Compensation Act.

---

Whatever pace you set, the fundamentals are constant: stay strictly within your non-medical scope, screen and train every caregiver, supervise every client at least every 90 days, document everything, treat clients and staff with respect, and keep your compliance airtight. Do those things well and growth follows.

---

*This guide is a general educational resource for prospective North Carolina home care agency owners. It is not legal, tax, or billing advice, and requirements, fees, and contacts change. Verify every step, figure, and phone number with DHSR, the NC Secretary of State, and the relevant payer before you rely on it. Provisions cited reflect North Carolina law and DHSR guidance at the time of writing.*